

Losing Lyumjev: changes since publication

Each change below gives where it goes in the live post and the text as it reads. The WordPress block HTML for each change, in the same order and headed by the same instruction, is in `changes_since_publication_wordpress.html`; paste a block into the code editor (Options, Code editor) at the place given. The full regenerated post is `diabettech-ultra-rapid-article-wordpress.html` in the same folder, if replacing the whole body is easier. Generated by `additions_since_publication.py` from the master article, compared with the live post as saved on 2026-10-04. New figures are marked as placeholders; the image files are in the Drive folder Diabettech article/figures.

1. Replacement paragraph

Where: Replace the paragraph "As MiniMed, Insulet, Tandem and CamDiab work to"... with the text below. It is the opening of the post.

Text:

As MiniMed, Insulet, Tandem and CamDiab work to bring fully closed loop, a system that needs no meal boluses at all, into wider use, the system that decides which insulins people are offered has conspired to take away a key part of that equation: the fastest-absorbed subcutaneous mealtime insulin available. This is a story of inertia, ineptitude and indifference.

2. Replacement paragraphs

Where: Replace the 3 paragraphs from the paragraph "Switzerland was the first. In March 2026 Lilly"... to the paragraph "Personally, I have used the fastest subcutaneous mealtime"... with the text below. It comes straight after the paragraph ending "a copy passed to it unofficially."

Text:

Norway lost two Lyumjev pens in October 2025, but Switzerland was the first country to lose its vial, cartridges and standard pen together. In March 2026 Lilly announced that it would stop selling selected presentations of its insulins in selected European countries before 2027, and Lyumjev, its ultra rapid insulin lispro, is on the list. The formal reason is "commercial". Closer to the ground, the reason given is that not many people used it.

So Europe is losing presentations of its fastest-absorbed subcutaneous insulin after years of surprisingly low and uneven use. The UK dispensing and formulary data in this article show that access policy is strongly associated with that use, and that for most of the period the price of the insulin explains very little of it. It cannot prove what drove Lilly's commercial decisions. But it raises an uncomfortable question: did health systems help create the low-demand market that the manufacturer is now retreating from? I think the answer is largely yes, and the rest of this article sets out why, and where the evidence stops.

Personally, I have [used](#) the fastest subcutaneous mealtime insulin available since each one reached the UK, some of the time on private prescription, because I read the pharmacology and the trials and decided that an insulin which starts working sooner was worth having. I have talked about it openly on this website. I built Boost, the open-source automated insulin delivery algorithm I develop, around the pharmacology of Lyumjev, and with it my own time in range (3.9 to 10 mmol/

L) was about 83% between 1 April and 31 August 2026, without announcing meals or bolusing for them ([calculation](#); the personal CGM data behind it are not published). This is a major reason for my concern.

3. Replacement paragraphs

Where: Replace the 3 paragraphs from the paragraph "Every injected rapid-acting insulin has the same problem."... to the paragraph "Lyumjev is the faster of the two. In"... with the text below. It comes straight after the heading "Why Lyumjev matters".

Text:

Every injected rapid-acting insulin has the same problem. Food starts raising glucose within minutes, and an injected insulin takes a good deal longer to get going. That is why we are told to inject 15 to 20 minutes before eating and many choose to do so considerably earlier than that. In practice many people do not wait. A [Spanish study](#) using connected pen caps recorded 775 evenings from 49 people, and in 52.6% of them (408 evenings) the dinner insulin went in up to 45 minutes after the start of the meal, as detected from the glucose trace.

The ultra-rapid insulins change the first half hour. Fiasp [appears in the blood](#) about five minutes earlier than NovoRapid. Lyumjev adds citrate and treprostinil to lispro, which open up the local blood vessels, and has [several times](#) Humalog's exposure in the first quarter of an hour and is typically faster still than Fiasp. In the meal test of [PRONTO-T1D](#), the rise in glucose an hour after eating was 1.55 mmol/L lower with Lyumjev than with Humalog, and a [meta-analysis](#) pooled reductions of 0.94 mmol/L in type 1 diabetes and 0.56 mmol/L in type 2, with little difference in HbA1c.

Lyumjev is the faster of the two. In a [crossover study of all four insulins](#), it reached half its early peak concentration six minutes before Fiasp, though its lower glucose after the meal against Fiasp, about 0.4 mmol/L at two hours, was numerical and not statistically significant. In closed-loop systems the gains from both are modest and depend on the algorithm. In the Cambridge CamAPS FX system, Fiasp [added no time in range](#) over standard aspart (though it reduced time below range), while Lyumjev [added 2.5 percentage points](#) over standard lispro, and a [pooled analysis of meals](#) found Lyumjev improved the four hours after breakfast where Fiasp made no measurable difference. In Medtronic systems, Fiasp added about 1.8 to 1.9 percentage points of time in range in two studies ([670G](#), [advanced hybrid closed loop](#)).

4. Replacement paragraph

Where: Replace the paragraph "It does not suit everyone. Site pain is"... with the text below. It comes straight after the paragraph ending "post-meal spikes compared with standard lispro.".

Text:

It does not suit everyone. Site pain is a common complaint, especially on pumps. In the [Lyumjev pump trial](#), infusion-site reactions occurred in 19.1% of people against 6.9% on Humalog, and of 29 people whose earlier public accounts I collected to illustrate experience, not to measure it, nine described pain or site reactions, seven of them with Lyumjev. "Burning, painful infusion sites that

left lumps several days after," one t:slim user wrote in March 2026. That is a reason to offer a choice: Lyumjev suits some people and hurts others.

5. Replacement paragraph

Where: Replace the paragraph "The UK was always going to struggle with"... with the text below. It comes straight after the paragraph ending "same arguments apply across national boundaries."

Text:

The UK was always going to struggle with this, as by far the most frequently prescribed rapid acting insulin comes from Novo (even though pricing for Novo and Lilly products has historically been similar) so it would have required a major marketing push to get Lyumjev in front of prescribers. In the year to July 2026, Lyumjev was 3.7% of the rapid-acting analogue insulin units dispensed in English primary care, six years after it arrived. Its share of lispro has grown every year, to 24% in 2026. Fiasp adds 13.9%, so the two ultra-rapid insulins together are 17.5% of units. It's notable that Fiasp is still a larger share than Humalog. Other European markets split differently. In France in 2025 Humalog was 27.3% of rapid-acting analogue units and Fiasp 11.1%; in Denmark, Novo Nordisk's home market, Fiasp was 11.0% and Humalog 1.1%. Standard-speed products make up the other 82.5% of units in English primary care, about 2.74 million prescription items a year.

6. Replacement paragraphs

Where: Replace the 3 paragraphs from the paragraph "Some of those gaps were made by the"... to the paragraph "This is an association between areas, and it"... with the text below. It comes straight after the paragraph ending "thirteen Scottish boards and Northern Ireland."

Text:

Some of those gaps were made by the process. From October 2020 the [Scottish Medicines Consortium](#) stopped requiring a submission for "an alternative formulation of an established medicine ... which costs the same per patient or less", which Lyumjev is, so there is no Scottish advice on it at all. The [All Wales Medicines Strategy Group](#) excluded Lyumjev from appraisal on the same grounds, as it had Fiasp. [Northern Ireland](#) adopts NICE decisions first, then the Scottish Consortium's, then the Welsh group's, and none of them had made one, so there was no recommendation for Northern Ireland to adopt through its usual sequence. Clinicians there can still prescribe outside the formulary or ask for an addition. A rule meant to spare same-priced medicines unnecessary paperwork left one with no decision anywhere, and no public record shows anyone, Lilly included, trying to close the gap.

Formulary restriction is associated with lower use. In England, where formularies left both ultra-rapid insulins open, the median ultra-rapid share was 26.8%; where they were second line, restricted to pregnancy or not listed, it was 9.5%. Birmingham and Solihull, the lowest in England, [allows Fiasp only in pregnancy](#) "after other insulins have been tried and failed" and does not list Lyumjev. Scotland shows the same direction: the two boards using the Highland Formulary, which lists both products without restriction, are at 26.2% and 23.5%, while the rest have a median of 5.2%. Across the 16 English integrated care boards, the rank correlation between how restrictive

the formulary is and ultra-rapid use was -0.72 (95% interval -0.91 to -0.35; permutation $p = 0.002$), and across Scotland's 13 boards -0.51.

Taken on its own, Lyumjev tells a narrower story. The more restrictive the formulary is about Lyumjev, the less of it is used, in England and across the UK ([figures](#)). Almost all of that comes from being left off. Where Lyumjev is not listed, its median share of lispro is 5.2% in England and 2.9% in Scotland; wherever English formularies list it, under any restriction, the median is 27% to 31%, with no clear gradient between the restrictions. Leaving it off the formulary goes with very low use. Putting it behind Humalog does not show up in these numbers.

This is an association between areas, and it does not demonstrate cause, but it is highly coincidental. The open group in England is only two integrated care boards; I classified the formularies myself after seeing the uptake figures; many formulary pages carry no date, so current wording is being compared with past prescribing; and none of it is adjusted for how many people use pumps, the reach of specialist centres, the local population or past habit. Wales goes the other way: its boards that require a specialist have higher uptake than its open ones (rank correlation +0.63), so formulary position explains part of the pattern in England and Scotland and plainly not in Wales. Northern Ireland dispenses Lyumjev at 1.6 times the English rate without listing it. Local specialist practice clearly matters as well. But a formulary that leaves an insulin off, or puts it behind the one most people are already on, makes it harder to prescribe.

7. Replacement paragraph

Where: Replace the paragraph "These data cannot show how often anyone was"... with the text below. It comes straight after the paragraph ending "can't easily transition them to biosimilars."

Text:

These data cannot show how often anyone was offered a faster insulin and declined it, perhaps because of site pain. As I've already mentioned, I'm reasonably sure that large swathes of the population simply weren't offered it. Among the small number of hospital prescriptions dispensed in the community, 0.13% of rapid-acting units, the ultra-rapid share is about twice the GP figure (35.6% against 17.6% on the same six-brand basis), yet most of it is still standard-speed. The much larger supply issued inside hospitals cannot be split by brand. What the data do show is that, through the years when the faster insulins carried little or no price premium, they stayed at under 6% of the rapid-acting insulin dispensed in England.

8. Replacement paragraphs

Where: Replace the 2 paragraphs from the paragraph "The biosimilar effort has been pointed at the"... to the paragraph "Long-acting insulin is where a switch asks nobody"... with the text below. It comes straight after the heading "Where the savings should come from".

Text:

The biosimilar effort has been pointed at the wrong insulin. In the year to July 2026, at list price and device for device, Fiasp cost English primary care exactly what NovoRapid cost, and Lyumjev exactly what Humalog cost. Moving someone from either older insulin to its faster version adds no acquisition cost at published list prices for the same device; the clinic time a switch takes is a

separate cost. The only money at stake is the saving given up by not moving them to Trurapi, which is £0.47 per 100 units cheaper than Fiasp. Lispro has no cheaper alternative left to protect, since Sanofi is withdrawing Admelog.

Long-acting insulin is where a switch does not mean accepting a slower insulin, though it is still a shared decision and a new pen to learn. Semglee, the cheapest glargine, lists at £2.00 per 100 units against £2.32 for Lantus and £2.35 for Abasaglar, the older biosimilar, and it is the same molecule with the same profile of action. Yet it is 4.8% of the glargine 100 dispensed in England. Moving 80% of Lantus and Abasaglar pens to it would save about £5.5 million a year. That is enough to pay for 14.5% of all the rapid-acting insulin dispensed in England to be Fiasp in place of Trurapi, and close to the £5.8 million, at most, that bringing England up to Wales's ultra-rapid share would forgo ([figures](#)).

9. Replacement paragraph

Where: Replace the paragraph "In France, Lilly applied for reimbursement and asked"... with the text below. It comes straight after the paragraph ending "campaign in the UK beyond it."

Text:

In France, Lilly applied for reimbursement and, according to the [assessment committee's transcript](#), claimed no added benefit over Humalog ("pas d'ASMR"), presenting the trials' comparable HbA1c and better post-meal glucose. The committee rated Lyumjev ASMR V, no improvement over Humalog, noting no demonstrated advantage in efficacy, tolerance or quality of life. In Germany Lilly agreed [rebate contracts](#) covering Humalog, Lyumjev and Abasaglar for "nearly all" people with statutory insurance.

10. Replacement paragraphs

Where: Replace the 2 paragraphs from the paragraph "I collected 31 public posts from people with"... to the paragraph "The sharpest reaction is from Switzerland, where Lyumjev"... with the text below. It comes straight after the heading "What the community is saying".

Text:

I collected 31 public posts from people with diabetes across Europe, and 11 statements from companies, regulators and patient groups, from 2025 and 2026. The posts were gathered to show the range of reactions. They are not a sample, so they say nothing about how common any view is.

The sharpest reaction is from Switzerland, where Lyumjev's main presentations went first. "Not cool that the best insulin for t1dm gets discontinued by the manufacturer - also without any sort of proper announcement" (Reddit, Switzerland, March 2026). "Why was my favorite insulin (Lyumjev) pulled from the Swiss market?" (Reddit, Switzerland, April 2026). "I am from Switzerland and currently use my very last Lyumjev pen" (Reddit, July 2026).

11. Replacement paragraphs

Where: Replace the 2 paragraphs from the paragraph "Fully closed loop works with the insulins already"... to the paragraph "So fully closed loop does not depend on"... with the text below. It comes straight after the paragraph ending "insulin its system is designed around."

Text:

Fully closed loop works with the insulins already available. Cambridge's CamAPS HX has run it with Fiasp in [adults with type 2 diabetes](#) and in [adolescents with type 1 diabetes](#), and with Lyumjev in [adults with type 1 diabetes](#). The one trial comparing Lyumjev with standard lispro in fully closed loop with unannounced meals found time in range of 49.3% against 39.9% in 17 people, over 8-hour inpatient sessions with one unannounced meal, a difference that did not reach statistical significance ($p = 0.072$). Its authors concluded that "Further advancements in faster-acting insulins are needed to alleviate the burden of pre-meal bolusing and enhance fully closed-loop performance in the future." As one person put it in September 2026: "every minute counts with a system working on a time offset."

So fully closed loop does not depend on Lyumjev. The 9.4-point difference in that trial would matter to someone living with it, but it came from a small, short study and was too imprecisely estimated to reach statistical significance. Insulin speed is still one of the few ways of reducing the delay these systems have to work around, and in Europe the fastest-absorbed subcutaneous insulin is being withdrawn country by country, with low use given as the reason by Lilly in Norway and, in the patient organisation's unofficial account, in Switzerland. Pump users can still fill reservoirs from Fiasp vials, as the German Diabetes Society has pointed out, but the options are narrowing as the systems that would benefit most arrive. The UK still has every Lyumjev presentation except the Tempo Pen.

12. Replacement paragraph

Where: Replace the paragraph "None of the reasons Lyumjev was so little"... with the text below. It comes straight after the heading "Where do we go from here?".

Text:

None of the reasons Lyumjev was so little used rests on evidence that the older insulins are better. They have, by default, been left in place: licensing trials built around equivalence on the measure everyone watches, guidance that names no product, formularies that left Lyumjev off altogether in a third of the UK areas I read, cost programmes that made a cheaper standard-speed insulin first choice once there was one, and a manufacturer that made little visible case for it.

13. Replacement paragraphs

Where: Replace the 3 paragraphs from the paragraph "What I want to see now is the"... to the paragraph "The cost of it all falls on the"... with the text below. It comes straight after the paragraph ending "one health board more than half."

Text:

What I want to see now is the diabetes charities and patient organisations of Europe standing up and being counted, and so far they have barely made a sound. Diabetes UK's page on insulin supply lists ten discontinuations and shortages and does not mention Lyumjev or the withdrawals across Europe, and Breakthrough T1D UK's site mentions Lyumjev only in its guide to connected pens. In France, which has already lost the cartridges, the last thing I can find on Lyumjev from the national federation of people with diabetes is a 2023 note about a temporary shortage. Germany's diabetesDE has nothing on it, and the German Diabetes Society's statement was practical advice on pump cartridge shortages ([search record](#)). Norway's diabetes association reported the withdrawals and quoted a professor reassuring readers that there were good alternatives to every insulin going. diabetesschweiz complained to Lilly, then passed on its own unofficial account of what it had been told: that hardly anyone used it. Reporting a withdrawal and relaying the manufacturer's reasons is the least an organisation can do, and these organisations exist to speak for the people who use these insulins. They should be demanding that the faster insulins stay on the market, that they are part of the fully closed-loop systems now in trials, that formularies stop parking them behind the insulin people are already on, and that clinicians offer them to everyone who might benefit and let each person decide for themselves whether to turn them down. When they stay quiet, the manufacturer's account of low demand is the only one anyone hears.

Is this a failure of the system? Yes, at every level, and nobody escapes the blame. The licensing trials were built to show the new insulins were no worse on HbA1c, and that was read as showing they were no better. NICE left its mealtime insulin advice as it stood in 2015, looked at it again in January 2026 and left it there. The Scottish and Welsh appraisal bodies wrote rules that left Lyumjev without a decision in either country, and Northern Ireland, which depends on them, inherited the gap. Formulary committees left it off in 13 of 38 areas, which is where its use is lowest, while finding a written place for Tresiba, which costs a third more than Lantus. Consultants told people who asked for a faster insulin that they were too well controlled to need one. Lilly priced Lyumjev the same as Humalog and then, as far as the public record shows, did almost nothing to make the case for it in the UK, and did not even tell the Swiss patient organisation that it was going. The charities, whose whole purpose is to make noise about exactly this, have barely raised their voices.

The cost of it all falls on people with diabetes who had less chance to make that choice. These data cannot show who was offered what, but they make it very hard to believe the pattern arose because almost everyone preferred the older insulins.

14. New sources 70 to 77

Where: Add at the end of the numbered Sources list, after entry 69.

Text:

1. Biosimilar rapid-acting insulin by country and year, with prices against the originator: https://github.com/tim2000s/rapid-insulin-prescribing/blob/master/international/output/RAPID_BIOSIMILAR_INTL.md
2. Ozer K, Cooper AM, Ahn LP, Waggonner CR, Blevins TC. Fast Acting Insulin Aspart Compared with Insulin Aspart in the Medtronic 670G Hybrid Closed Loop System in Type 1 Diabetes: An Open Label Crossover Study. *Diabetes Technol Ther* 2021;23(4):286-292. <https://doi.org/10.1089/dia.2020.0500>

3. Lee MH, Paldus B, Vogrin S, Morrison D, Zaharieva DP, Lu J, et al. Fast-Acting Insulin Aspart Versus Insulin Aspart Using a Second-Generation Hybrid Closed-Loop System in Adults With Type 1 Diabetes: A Randomized, Open-Label, Crossover Trial. *Diabetes Care* 2021;44(10):2371-2378. <https://doi.org/10.2337/dc21-0814>
4. National Institute for Health and Care Excellence. Type 1 diabetes in adults (NG17): surveillance review of 30 January 2026, decision not to update; recommendations 1.7.10 to 1.7.12 on mealtime insulin unchanged from 2015. <https://www.nice.org.uk/guidance/ng17>
5. Tandem Diabetes Care. t:slim X2 insulin pump with Control-IQ technology, technical user guide outside the US (AW-1020085, 2 September 2026), listing Lyumjev as a compatible insulin. <https://assets.ctfassets.net/k5s5iz8jouwn/74Y30BhsHQ7oRonVR5PvF/4e023ed1c21d1fdcb14d34d6546db030/technical-user-guide-tslimx2-control-iq-7-10-4-en-ous-AW1020085.pdf> ; Insulet. Omnipod 5 UK important safety information. <https://www.omnipod.com/en-gb/safety> (both accessed 1 October 2026)
6. diabetesschweiz. Email reply of 10 March 2026 as posted publicly by its recipient on Reddit (r/diabetes). https://www.reddit.com/r/diabetes/comments/1rlmdui/lyumjev_apparently_discontinued_ausser_handel_in/o9nkdrx/ (accessed 1 October 2026)
7. Haute Autorité de Santé. Commission de la Transparence, session of 3 June 2020, Lyumjev (CT-18474): "Le laboratoire demandait un SMR important, pas d'ASMR". https://www.has-sante.fr/upload/docs/application/pdf/2020-07/lyumjev_03062020_transcription_ct18474.pdf
8. Formulary position and uptake: ultra-rapid analysis (England and UK) and Lyumjev-specific analysis, with the author's time-in-range calculation: https://github.com/tim2000s/rapid-insulin-prescribing/blob/master/output/formulary/UK_SUMMARY.md ; https://github.com/tim2000s/rapid-insulin-prescribing/blob/master/output/formulary/LYUMJEV_SUMMARY.md ; https://github.com/tim2000s/rapid-insulin-prescribing/blob/master/author_tir/RESULT.md